

1. Administrative & Study Identification

Full Study Title: A Phase III, Randomized, Open-Label Study Evaluating the Efficacy and Safety of Divarasib and Pembrolizumab Versus Pembrolizumab and Pemetrexed and Carboplatin or Cisplatin in Patients With Previously Untreated, KRAS G12C-Mutated, Advanced or Metastatic Non-Squamous Non-Small Cell Lung Cancer
Short Title/Acronym: Krascendo 2 **Protocol Number:** C045042 **NCT Number:** NCT06793215 **Sponsor Name:** Hoffmann-La Roche (Roche)
Investigational Product: Divarasib (KRAS G12C inhibitor) and Pembrolizumab (Immune checkpoint inhibitor) **Phase of Development:** Phase III **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective * **Primary Objective:** To evaluate the efficacy and safety of first-line treatment with divarasib and pembrolizumab versus pembrolizumab and chemotherapy (pemetrexed + carboplatin/cisplatin) in adult patients with advanced or metastatic KRAS G12C-mutated non-squamous non-small cell lung cancer (NSCLC). * **Secondary Objectives:** To evaluate confirmed objective response rate (ORR), duration of response (DOR) by Blinded Independent Central Review (BICR), changes in patient-reported symptoms and functioning from baseline to Cycle 5, and overall safety profile.

2.2 Background & Rationale KRAS G12C mutations are found in approximately 12% of NSCLC cases. The current standard recommended first-line treatment for patients with advanced or metastatic non-squamous KRAS G12C-mutated NSCLC is immunotherapy (most commonly pembrolizumab) with or without platinum-based chemotherapy. However, there is an unmet clinical need in this patient population for more efficacious, chemo-free therapies with tolerable and manageable safety profiles. Divarasib is a potent KRAS G12C inhibitor that has shown promising efficacy and safety as monotherapy; combining it with pembrolizumab is hypothesized to be an effective first-line chemo-free treatment option.

3. Study Design & Methodology

3.1 Design Framework * **Study Type:** Interventional * **Control Type:** Active Comparator (Pembrolizumab + Pemetrexed + Carboplatin/Cisplatin) * **Blinding:** Open-label * **Randomization:** Randomized

3.2 Planned Enrollment & Duration * **Target N\$:** 600 * **Trial Status:** RECRUITING * **Number of Sites:** Multicenter, Global * **Estimated Study Start:** Ongoing * **Estimated Primary Completion:** Late 2029 (results expected in approx. 3 years and 8 months)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years old. 2. Histologically or cytologically confirmed diagnosis of advanced or metastatic non-squamous NSCLC that is not eligible for curative surgery and/or definitive chemoradiotherapy. 3. Documentation of the presence of a KRAS G12C mutation and known PD-L1 expression status in tumor tissue. 4. Measurable disease per RECIST v1.1. 5. No prior systemic treatment for advanced or metastatic NSCLC. 6. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1. 7. Eligible to receive a platinum-based chemotherapy regimen.

4.2 Exclusion Criteria 1. Known concomitant second oncogenic driver with available targeted treatment. 2. Symptomatic, untreated, or actively progressing central nervous system (CNS) metastases, or history of leptomeningeal disease. 3. Prior treatment with KRAS G12C inhibitors or pan-KRAS/RAS inhibitors. 4. History of inflammatory bowel disease (e.g., Crohn's disease or ulcerative colitis), active bowel inflammation, chronic diarrhea, or malabsorption syndrome. 5. History of idiopathic pulmonary fibrosis, organizing pneumonia, drug-induced pneumonitis, or idiopathic pneumonitis.

5. Intervention & Treatment Plan

5.1 Investigational Regimen * **Dosage/Frequency:** Divarasib (oral) combined with Pembrolizumab (IV) given in standard cycles. * **Route of Administration:** Oral (Divarasib) and Intravenous (Pembrolizumab). * **Duration of Treatment:** Until disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy * **Permitted:** Standard supportive care for NSCLC symptoms. * **Prohibited:** Systemic immunosuppressive or immunostimulatory medications (including CD137 agonists), medications well known to prolong the QT interval, and therapeutic oral or IV antibiotics within 2 weeks prior to randomization.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Tumor Specimen Collection, Baseline Scans (CT/MRI) Baseline
Day 0	Randomization, First Dose	Interim Assessment 1	

First 72 Weeks | Tumor assessments every 6 weeks (\$\pm\$ 7 days) |
| **Interim Assessment 2** | Post 72 Weeks | Tumor assessments every
9 weeks (\$\pm\$ 7 days) | | **End of Study** | TBD | Final Vital
Signs, Exit Interview, IP Accountability, Survival Follow-up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint * **Definition:** Progression-Free Survival (PFS) and Overall Survival (OS). * **Measurement Method:** Evaluated by Blinded Independent Central Review (BICR) per RECIST v1.1.

7.2 Secondary & Exploratory Endpoints * Confirmed Objective Response Rate (ORR) by BICR. * Duration of Response (DOR) by BICR. * Changes in patient-reported symptoms and functioning from baseline to Cycle 5 (assessed via questionnaires).

8. Safety & Adverse Event Reporting

- **Adverse Event (AE) Monitoring:** Standard collection focusing on GI toxicities (diarrhea), pneumonitis, and hepatotoxicity.
 - **Serious Adverse Events (SAEs):** Routine 24-hour reporting timeline per standard pharmacovigilance practices.
 - **Data Monitoring Committee (DMC):** External independent committee reviewing unblinded safety and efficacy data.
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9. Statistical Analysis Plan (SAP) Summary

- **Analysis Populations:** Intent-to-Treat (ITT) population for primary efficacy endpoints (PFS/OS). Safety Set for all subjects receiving at least one dose of study drug.
 - **Sample Size Justification:** Powered to detect a statistically significant improvement in PFS and OS for Divarasib + Pembrolizumab compared to standard of care.
 - **Handling of Missing Data:** Censoring rules applied for PFS/OS per standard FDA/EMA oncology guidelines.
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10. Quality Assurance & Regulatory Compliance

- **GCP Compliance:** This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
 - **Monitoring Plan:** Routine onsite and remote monitoring for data integrity, source data verification, and protocol compliance.
 - **Audit Trail:** eCRF locking and electronic data clarification forms.
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11. Study Identifiers & Governance

- **Primary ID:** C045042
 - **Posting ID:** 1096784258430081179
 - **Registry Number:** NCT06793215
 - **Sponsor/Lead Organization:** Hoffmann-La Roche (Roche)
 - **Company:** Roche
 - **Study Title:** Krascendo 2
 - **Official Regulatory Title:** A Phase III, Randomized, Open-Label Study Evaluating the Efficacy and Safety of Divarasib and Pembrolizumab Versus Pembrolizumab and Pemetrexed and Carboplatin or Cisplatin in Patients With Previously Untreated, KRAS G12C-Mutated, Advanced or Metastatic Non-Squamous Non-Small Cell Lung Cancer
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12. Clinical Framework (Oncology Specific)

- **Primary Condition:** Non-Small Cell Lung Cancer (NSCLC), Advanced/Metastatic Non-Squamous
 - **Disease Areas:** Non-Small Cell Lung Cancer, KRAS G12C Lung Cancer
 - **Preferred UMLS Name:** KRAS G12C Positive Non-Small Cell Lung Carcinoma
 - **Diagnosis Threshold:** Documentation of KRAS G12C mutation and PD-L1 expression
 - **Medical Methodology:** Targeted Therapy + Immunotherapy vs. Chemotherapy + Immunotherapy
 - **Optimization Target:** Improved Progression-Free Survival (PFS) and Overall Survival (OS)
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13. Study Procedures & Methodology

- **Management Pathway:** Standard oncological pathway for metastatic NSCLC.
 - **Education Protocol (HCP):** Site initiation visits and RECIST v1.1 tumor assessment training.
 - **Education Protocol (Patient):** Patient-reported outcome questionnaire training (PROs assessed up to Cycle 5).
 - **Quality Audit Mechanism:** Blinded Independent Central Review (BICR) for all radiological tumor assessments.
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14. Observation & Follow-Up Schedule

- **Total Duration:** Until disease progression or death
- **Onsite Visit Cadence:** Aligned with treatment cycles
- **Radiological Monitoring Frequency:** Every 6 weeks for the first 72 weeks, then every 9 weeks
- **Checklist Review Interval:** Continuous safety monitoring prior to each dosing cycle

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					